TEST REPORT

Name	: **Mr. AZMATH ULLAH**	Registered on	: 23-Dec-2025 06:56
Age/Gender	: **50 Years / Male**	Collected on	: 23-Dec-2025 06:59
Registration ID	: **250150062521**	Released on	: 23-Dec-2025 11:23
Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Whole Blood - EDTA	Regn Centre	: **Santosh Nagar-15**

GLYCATED HAEMOGLOBIN (HbA1C)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Glycated Haemoglobin	: **10.0**	%	Non Diabetic : < 5.7 Pre - Diabetic Range : 5.7 - 6.4 Diabetic Range : >/= 6.5

Method: High performance liquid chromatography

Interpretation / Comments:

Reference Range as per American Diabetes Association (ADA) 2025 Guidelines:

- HbA1c is used to diagnose Diabetes, monitor glycemic control, evaluate the need to change therapy and predict the development of microvascular complications.
- The Glycemic target for non-pregnant adults with diabetes is <7.0%. Lowering A1c value below or around 7.0% has been shown to reduce microvascular complications and macrovascular disease.
- Trends in HbA1C values, at interval of 3 months are a better indicator of Glycemic control than a single test.
- Glycated hemoglobin is proportional to mean plasma glucose level during the previous 6-12 weeks.
- HbA1c result may not correlate with the blood glucose level in presence of Hemoglobin variants, conditions that affect red cell turnover, Systemic Inflammatory Diseases, Chronic renal failures and liver diseases, recent blood transfusions and hypertriglyceridemia. Clinical Correlation is advised in interpretation of low Values.
- ADA guidelines 2025 is being followed.

MC-6432

DR. HAFSA SALEEM
MD PATHOLOGY
Registration No: TSMC/FMR/87485

Chromatogram Report

G11 V03.11 2025-12-23 11:04:06

ID 2250150062521
Sample No. 2025122311020056 SL 0004 - 04
Patient ID
Name
Comment

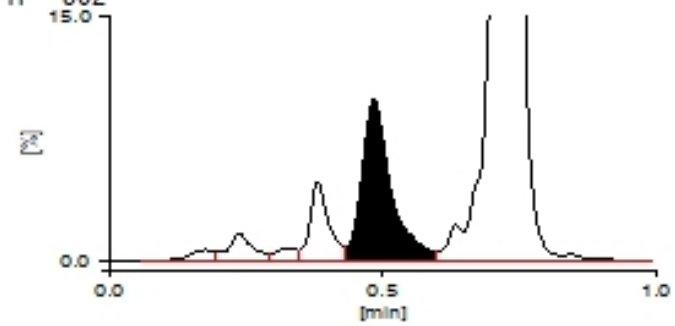
CALIB(N) Y =1.1698X + 0.5732

Name	%	Time	Area
FP			
A1A	0.5	0.17	6.03
A1B	1.2	0.24	15.24
F	0.4	0.32	5.74
LA1C+	2.6	0.38	32.89
SA1C	10.0	0.48	105.11
A0	87.5	0.73	1114.69
H-VAR			
		Total Area	1279.70

HbA1c 10.0 %

HbF 0.4 %

TP 502



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* Suggested Clinical Correlation, If Necessary Kindly Discuss with signatory

TEST REPORT

Name	: **Mr. AZMATH ULLAH**	Registered on	: 23-Dec-2025 06:56
Age/Gender	: **50 Years / Male**	Collected on	: 23-Dec-2025 09:31
Registration ID	: **250150062521**	Released on	: 23-Dec-2025 15:08
Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Fluoride Plasma	Regn Centre	: **Santosh Nagar-15**

POST LUNCH GLUCOSE

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Post Lunch Glucose *Method: Hexokinase*	:	**174**	mg/dL	Normal : <140 Impaired Glucose Tolerance : 140 - 199 Diabetes : >/= 200

Interpretation / comments:

- Discordant plasma glucose level (i.e. post prandial plasma glucose level lower than fasting plasma glucose level) can be observed, which could be due to the glycemic index and the response to the food consumed, increased insulin secretion, effects of oral hypoglycemic drugs and insulin treatment, defective absorption, insufficient dietary intake, endocrine disorders and reactive hypoglycemia etc.
- The diagnosis of diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.
- Very high glucose levels (>450 mg/dL in adults) may result in diabetic ketoacidosis & is considered critical.
- ADA (American Diabetes Association) guidelines – 2025 is adopted for the evaluation of diabetes status.

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TEST REPORT

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Age/Gender	: **50 Years / Male**	Collected on	: 23-Dec-2025 06:59
Registration ID	: **250150062521**	Released on	: 23-Dec-2025 10:52
Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Serum	Regn Centre	: **Santosh Nagar-15**

LIPID PROFILE

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Triglycerides *Method: Glycerol Phosphate Oxidase (GPO), Peroxidase (POD)*	:	107	mg/dL	Desirable level : < 150 Borderline : 150 - 199 High : 200 - 499 Very High : > / = 500
Total Cholesterol *Method: Cholesterol Oxidase Peroxidase (CHOD-POD)*	:	151	mg/dL	Desirable : < 200 Borderline high : 200 - 239 High : > / = 240
LDL Cholesterol *Method: Calculated*	:	84	mg/dL	Optimal : < 100 Near optimal : 100 - 129 Borderline high : 130 - 159 High : 160 - 189 Very high : >/= 190
HDL Cholesterol *Method: Enzymatic Immunoinhibition*	:	46	mg/dL	Desirable level : >/= 60 Optimal : 40 - 59 Undesriable : < 40
VLDL Cholesterol *Method: Calculation*	:	21	mg/dL	< 30
Total Cholesterol / HDL Cholesterol Ratio *Method: Calculation*	:	3.3		Low risk : 3.3 - 4.4 Average risk : 4.5 - 7.1 Moderate risk : 7.2 - 11.0
LDL Cholesterol / HDL Cholesterol Ratio *Method: Calculation*	:	1.8		Desirable level : 0.5 - 3.0 Borderline risk : 3.0 - 6.0 High risk : > 6.0

Interpretation / Comments :

- Lipid profile is a panel of blood tests that serves as an initial broad medical screening tool for abnormalities in lipids, the results of this tests can identify certain genetic diseases and can determine approximate risks for cardiovascular disease, certain forms of pancreatitis and other diseases.

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Customer Care ✆ 040-2100 0000
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TEST REPORT

Name	: **Mr. AZMATH ULLAH**	Registered on	: 23-Dec-2025 06:56
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Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Urine	Regn Centre	: **Santosh Nagar-15**

CUE (COMPLETE URINE EXAMINATION)

TEST NAME	RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Physical Examination			
Colour	: Pale Yellow		Pale Yellow
Method: Physical Examination			
Appearance	: Clear		Clear
Method: Physical Examination			
Specific Gravity	: 1.005		1.003 - 1.030
Method: Ion-Exchange			
Chemical Examination			
Reaction/pH	: 5.0		4.6 - 8
Method: Reagent Strip Method / Double Indicator			
Protein	: Nil		Neg-Trace
Method : Reagent Strip/Protein error of Indicator			
Glucose	: **Present (+++)**		Negative
Method: GOD-POD			
Urobilinogen	: Normal		Normal
Method : Reagent Strip/Modified Ehrlich reaction			
Bilirubin	: Negative		Negative
Method: Diazonium			
Ketones	: Negative		Negative
Method : Reagent Strip/Nitroprusside reaction			
Nitrites	: Negative		Negative
Method: Reagent strip / Diazonium			
Microscopic Examination			
Pus Cells	: 1-2	cells / HPF	0 - 5
RBC	: 0-1	cells / HPF	0 - 2
Epithelial Cells	: 1-2	cells / HPF	0 - 5
Casts	: Nil		Absent
Crystals	: Nil		Absent
Method	: Reagent Strips, Wave length reflectance method, Refractive index, Flow Digital Imaging technology using APR and Microscopy.		

MC-6432

DR. RAVIKUMAR T
MD PATHOLOGY
Registration No: KMC/FMR/84462

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TEST REPORT

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Age/Gender	: **50 Years / Male**	Collected on	: 23-Dec-2025 06:59
Registration ID	: **250150062521**	Released on	: 23-Dec-2025 11:09
Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Fluoride Plasma	Regn Centre	: **Santosh Nagar-15**

FASTING PLASMA GLUCOSE

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Fasting Plasma Glucose *Method: Hexokinase*	:	**135**	mg/dL	Normal : 70 - 100 Impaired Fasting Glucose : 101 - 125 Diabetes : >/=126

Interpretation / Comments :

- The diagnosis of Diabetes requires a fasting plasma glucose of > or = 126 mg/dL and/or a random / 2 hr post glucose value of > or = 200 mg/dL on at least 2 occasions.
- Very high glucose levels (>450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.
- ADA (American Diabetes Association) guidelines – 2025 is adopted for the evaluation of diabetes status.

MC-6432

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Ref. By	: Dr RIYAZ	Printed on	: 23-Dec-2025 15:24
Sample Type	: Serum	Regn Centre	: **Santosh Nagar-15**

URIC ACID

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Uric Acid	:	5.1	mg/dL	3.5 - 7.2

Method: Uricase-Peroxidase

Interpretation / Comments :

- Useful for monitoring therapeutic management of gout and chemotherapeutic treatment of neoplasms.

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Sample Type	: Serum	Regn Centre	: **Santosh Nagar-15**

UREA

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Urea	:	23	mg/dL	17 - 43
Method: Urease/GLDH				

Interpretation / Comments :

- In conjunction with serum creatinine,urea level aids in differential diagnosis of pre-renal,renal and post-renal hyperuremia.

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Sample Type	: Serum	Regn Centre	: **Santosh Nagar-15**

CREATININE

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Creatinine	:	**0.6**	mg/dL	0.7 - 1.2
Method: Jaffe Kinetic IDMS traceable				
e-GFR (Glomerular Filtration Rate)	:	122.1	ml/min/1.73 m²	>/= 90
Method: Calculation, CKD EPI equation				

Interpretation / Comments :

- Serum Creatinine is useful in the diagnosis of renal insufficiency and is more specific and sensitive indicator of renal disease than serum Urea /BUN.
- Use of simultaneous Urea / BUN and creatinine levels provide more information in the diagnosis of renal insufficiency.
- Amount of Creatinine produced in the body is relatively constant (unlike Urea) and is primarily a function of muscle mass (Creatinine being a waste product formed in muscles.).
- Age, gender, Race, Muscle mass, Exercise, Certain Drugs, diet, Dehydration are amongst the physiological factors affecting serum creatinine concentration.
- GFR (Glomerular Filtration Rate) is generally considered the best index of overall kidney function. Repeated determination in conjunction with creatinine assay establish whether the patient has stable or progressive disease.
- The CKD-EPI equation is the most widely used IDMS traceable equations for estimating GFR in patients above 18 years of age. This equation includes variables for age and gender, and it may be observed that Kidney may be involved despite a serum creatinine concentration appearing to be within or just above the Biological Reference Interval. The results of e-GFR by CKD-EPI equation are normalized to 1.73 m2 body surface area. CKD-EPI equation is not valid for individuals under 18 years of age.
- Estimates for GFR based on serum creatinine will be less accurate for patients at the extremes of muscle mass (such as frail elderly, critically ill, cancer patients) and also those with unusual diets, sudden acute renal failure, patients on dialysis and patients with severe liver disease.
- Confirmatory tests with exogenous measured GFR or directly measured creatinine clearance should be performed for such individuals.
- The influence of creatinine measurement imprecision at low creatinine conecentrations (and hence high e-GFR) has a possible contribution to the variability at higher e-GFR values.

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Sample Type	: Serum	Regn Centre	: **Santosh Nagar-15**

ELECTROLYTES

TEST NAME		RESULT	UNIT	BIOLOGICAL REFERENCE INTERVAL
Sodium	:	138	mmol/L	136 - 146
Method: Indirect ISE				
Potassium	:	4.6	mmol/L	3.5 - 5.1
Method: Indirect ISE				
Chlorides	:	102	mmol/L	101 - 109
Method: Indirect ISE				

Interpretation / Comments :

- Sodium : Levels of sodium when evaluated with electrolytes aid in assessing acid base balance, water balance and water intoxication.
- Potassium : Useful in evaluation of electrolyte balance, cardiac arrhythmia, muscular weakness, hepatic encephalopathy and renal failure.
- Chloride : Useful when assayed along with sodium, potassium and bicarbonate in assessment of electrolyte, acid base and water balance.

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---------- End of Report ----------